

Mental health problems

» What are they?

Conditions as diverse as bipolar disease, depression, psychosis, neurosis, self harming, obsessive-compulsive disorder, addictions, and eating disorders.

» Risks to your pregnancy, labor, and baby

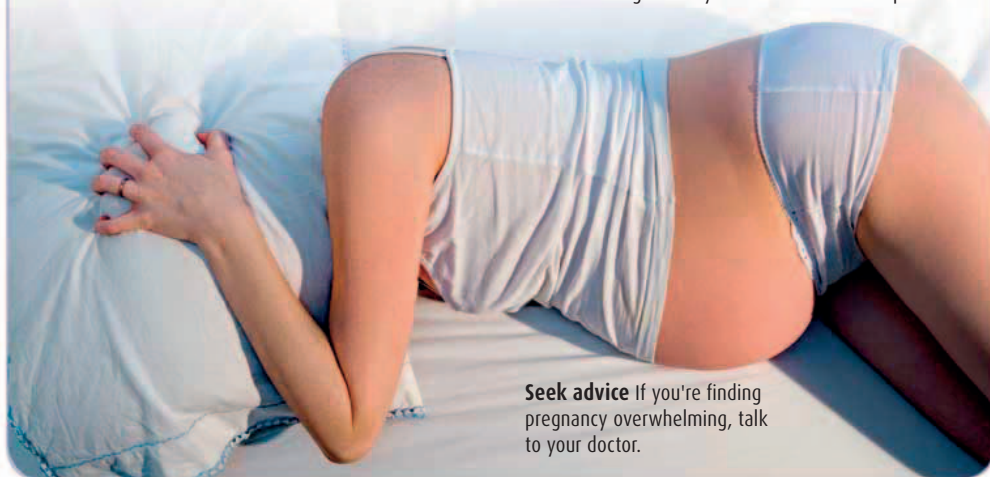
Medications for mental health problems can affect the health of your unborn baby. You are also at increased risk of developing prenatal depression and postpartum depression. You may feel unable to take care of yourself and unwilling or unable to nurture your unborn baby; if you have an eating disorder, you may restrict your baby's access to important nutrients in utero. Studies show that babies born to women suffering from mental health problems are also at greater risk of developing depression-related conditions themselves in later life.

» Medicines and tests

Do not change or stop taking your medication without first consulting your doctor. He or she will need to make a full assessment of your condition and will devise a medication maintenance program that you can use throughout your pregnancy to minimize the risks for your baby, while maintaining your health.

» Effects of pregnancy on your condition

Scientific studies show that the anxiety associated with being pregnant, as well as the need to reduce medication can put women with a history of mental illness at greater risk of psychosis or depression during pregnancy. However, if your condition is under control before you're pregnant, there is evidence to suggest that with the right level of support and medicine, you are not at a significantly increased risk of a relapse.



Seek advice If you're finding pregnancy overwhelming, talk to your doctor.

Epilepsy

» What is it?

A disorder of the electrical signals in the brain, epilepsy causes repeated seizures.

» Risks to your pregnancy, labor, and baby

You might experience nausea and vomiting, anemia, pregnancy-induced hypertension, or preeclampsia. Seizures don't harm your baby unless they're tonic clonic, clustered, or last for more than 30 minutes—all of which can starve a fetus of oxygen. Injuring yourself during a seizure could also harm the baby. Don't shower or bathe in pregnancy, or bathe your newborn, with the door locked or when there is no one in the house. There is a higher risk of premature or prolonged labor, and needing a C-section. After birth, your baby may have withdrawal from epilepsy medicines (AEDs) and may need an extra dose of vitamin K to help with blood clotting.

» Medicines and tests

Take your medication unless your neurologist advises you not to. Some AEDs increase the risk of newborn congenital abnormalities, but 90 percent of women with epilepsy have healthy babies. The risks to your baby if you have a seizure are greater. A blood test can assess the levels of medicine in your body since increased blood volume in pregnancy means your usual dosage may be too diluted. Change your dosage only under medical advice. Talk to your doctor about increasing folic acid intake since some medicines inhibit its absorption (though folic acid can inhibit the efficacy of your medication). You may need a vitamin K supplement from week 36, since epilepsy medicines can affect blood clotting. You may need to avoid opioid analgesics in labor.

» Effects of pregnancy on your condition

Most women with epilepsy find that there is no change in pregnancy. Sometimes symptoms improve; in other cases seizures increase, which may continue after birth. If seizures increase, ask your neurologist to review your medication.

Congenital/acquired heart disease

» What are they?

Congenital heart disease (CHD) is a condition you're born with. Acquired heart disease (AHD) is usually caused by rheumatic fever, Kawasaki disease, or obesity.

» Effects on your pregnancy, labor, and baby

Your baby is at a slightly increased risk of developing congenital heart defects (even if your own heart disease is acquired). There may be restricted oxygen to your baby, which can cause low birth weight, premature birth, or stillbirth. You are at increased risk of fluid on your lungs—see your doctor if you experience any chest pains or abnormally increased coughing. A recent study found that women with congenital heart disease were more likely to have a cesarean section and longer hospital stays. You'll be encouraged to give birth lying on your left-hand side, as this reduces the pressure on your major blood vessels. Your breathing during the final stage of labor will be carefully guided so that you don't hold your breath. Your baby will be attached to a fetal heart monitor throughout labor, and you will probably need your own continual ECG reading. You may need to have antibiotics during labor in order to reduce your risk of heart infection.

» Medicines and tests

Your doctor may suggest increased blood pressure checks as a noninvasive measure of your circulation function. Anticoagulant medication can cross the placenta and cause your baby's blood to thin dangerously. Your doctor will advise you on keeping you and your baby healthy.

» Effects of pregnancy on your condition

Your weight, diet, and exercise levels will be monitored over your pregnancy to ensure good nutrition and steady weight gain; excess weight during pregnancy can put strain on your heart. The opening of your blood vessels is a natural side effect of increased progesterone. This can help blood flow to your heart, but increased blood volume and any shortening of breath can strain your heart.

Systemic lupus erythematosus

» What is it?

Sometimes known simply as "lupus," or SLE, this autoimmune disorder causes the immune system to attack healthy tissue in its own body by mistake.

» Effects on your pregnancy, labor, and baby

You are at increased risk of preeclampsia, premature labor, miscarriage, and stillbirth. If SLE has affected your kidneys, you may be at increased risk of high blood pressure. If SLE antibodies cross the placenta, this can cause a harmless skin rash in the baby at birth, and more seriously, your baby